BASELINE → TREATMENT → FOLLOW-UP

Checks throughout treatment

Relative stages; no dates need to be entered. Ask who arranges and reviews each check.

Check	Before starting	Cycle 1 · Day 1	Cycle 1 · Day 22	Later cycles · Day 1	Later cycles · Day 22	Completion / follow-up
Treatment	Confirm indication and plan	Nivolumab + ipilimumab	Nivolumab alone	Nivolumab + ipilimumab	Nivolumab alone	Agree stop plan and continuing care
Blood count, kidney and liver	Baseline results	Review before treatment	Clinical review; additional tests as indicated	Review before every cycle	Clinical review; additional tests as indicated	Follow unresolved toxicity
Hormones and immune symptoms	Glucose, TFT; hepatitis screen; cortisol if indicated	Glucose/TFT before cycle and symptom review	Review symptoms; extra tests if needed	Glucose/TFT before cycle and symptom review	Review symptoms; extra tests if needed	Vigilance at least five months after last dose
Support medicines	Carry treatment details and the immune alert card. Describe your usual bowel pattern, breathing, energy, skin and hormone symptoms, and tell the team about autoimmune illness, infections and all medicines.	Low-risk visit support	No routine minimal-risk prophylaxis	Low-risk support; no chemotherapy prophylaxis	Minimal-risk support; no routine prophylaxis	Confirm medicines and steroid taper if used
Response and preferences	Discuss goals and alternatives	Ask about symptoms and priorities	Review treatment experience	Consultant-directed scans and benefit review	Report new concerns between visits	Agree responsible team and symptom support

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00792 v2 · official regimen

THE WHOLE TREATMENT JOURNEY

Nivolumab and ipilimumab for pleural mesothelioma: treatment and checks

First-line unresectable malignant pleural mesothelioma in adults.

Each cycle lasts 42 days: nivolumab and ipilimumab on Day 1, then nivolumab alone on Day 22. Nivolumab is every three weeks and ipilimumab every six weeks. This regimen contains no chemotherapy. Continue while appropriate until progression, unacceptable toxicity or up to 24 months without progression. Your team reviews the plan and may delay or stop treatment.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Cycle 1 · Day 1	Cycle 1 · Day 22	Later cycles · Day 1	Later cycles · Day 22	Completion / follow-up
Treatment NCCP 00792	Confirm indication and plan	Nivolumab + ipilimumab	Nivolumab alone	Nivolumab + ipilimumab	Nivolumab alone	Agree stop plan and continuing care
Blood count, kidney and liver NCCP 00792	Baseline results	Review before treatment	Clinical review; additional tests as indicated	Review before every cycle	Clinical review; additional tests as indicated	Follow unresolved toxicity
Hormones and immune symptoms NCCP 00792	Glucose, TFT; hepatitis screen; cortisol if indicated	Glucose/TFT before cycle and symptom review	Review symptoms; extra tests if needed	Glucose/TFT before cycle and symptom review	Review symptoms; extra tests if needed	Vigilance at least five months after last dose
Support medicines NCCP 00792	Carry treatment details and the immune alert card. Describe your usual bowel pattern, breathing, energy, skin and hormone symptoms, and tell the team about autoimmune illness, infections and all medicines.	Low-risk visit support	No routine minimal-risk prophylaxis	Low-risk support; no chemotherapy prophylaxis	Minimal-risk support; no routine prophylaxis	Confirm medicines and steroid taper if used
Response and preferences NCCP 00792	Discuss goals and alternatives	Ask about symptoms and priorities	Review treatment experience	Consultant-directed scans and benefit review	Report new concerns between visits	Agree responsible team and symptom support

What each check is for

1 Baseline

Before first infusion

Confirm exact eligibility

Ask / act: Review results, consent, medicines and contacts.

Sources: NCCP 00792

2 Day 22

Three weeks after Day 1

Different active medicines

Ask / act: No ipilimumab on Day 22; confirm support plan.

Sources: NCCP 00792

3 Continuation

Later 42-day cycles

Review benefit and toxicity

Ask / act: Apply joint immune hold rules and review duration.

Sources: NCCP 00792

4 End of treatment

At progression/toxicity or up to 24 months

Late immune effects can occur

Ask / act: Document follow-up and urgent contacts.

Sources: NCCP 00792

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-29 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.27